EXPECT™

AI-Augmented Pelvic Floor Physical Therapy

A Pilot Proposal for the Utah Office of Artificial Intelligence Policy (Regulatory Sandbox)

Confidential — Contains Business Confidential Information | February 2026

Physician-Approved Maternal Care

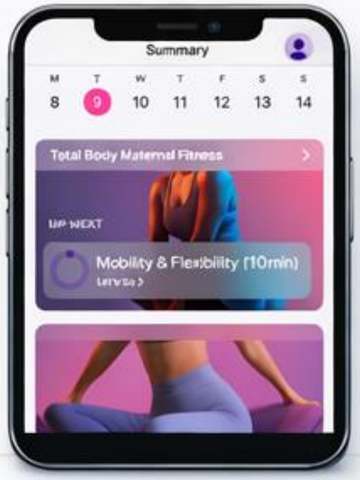
Prenatal Fitness

OB/GYN-Approved

Safe workouts for expecting moms

Postnatal Fitness

OB/GYN-Approved



Recovery & strength for new moms

Pelvic Floor PT

Telehealth

Licensed PT pelvic therapy

Trusted by Top Institutions.

Active CRADA Partner

IRB-Approved Research

Member Organization

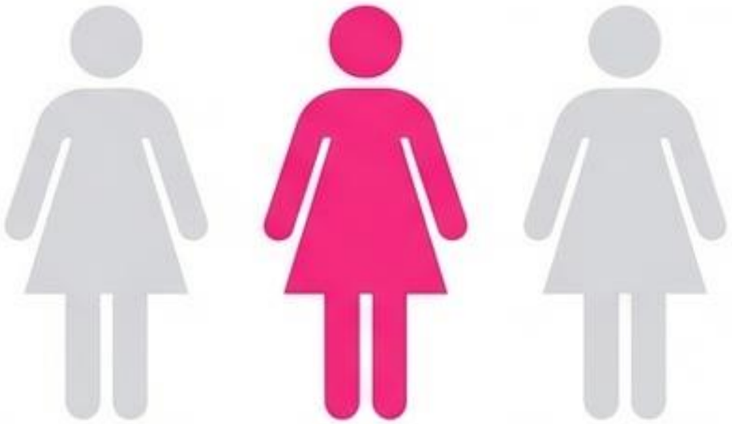
Active Contract

Operating with federal-level clinical rigor and industry-standard ethics.

A Silent Epidemic in a High-Volume Maternity State

of U.S. women affected by urinary incontinence.

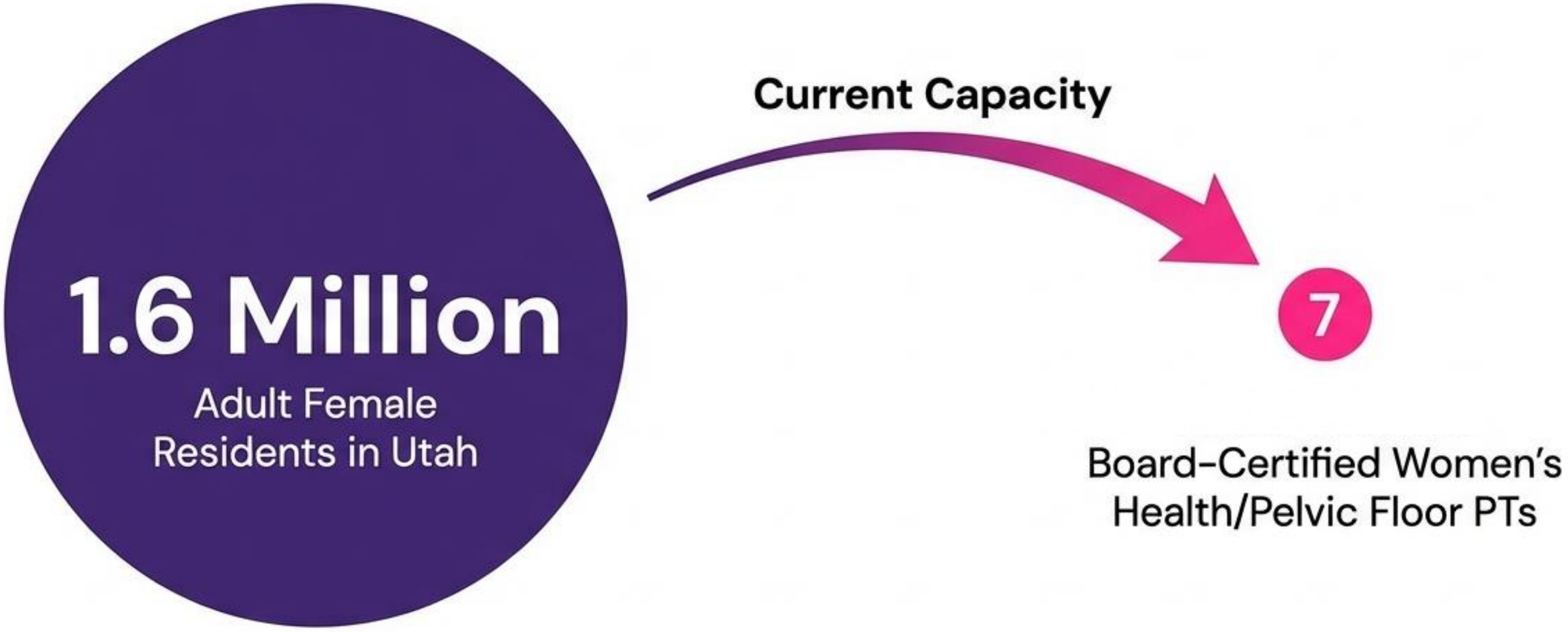
1 in 3



women in Utah experience incontinence after childbirth.

Sources: Milsom et al., 2019; Lee et al., 2021; Patel et al., 2022; Che et al., 2025; Thom & Rortveit, 2010

The Math Does Not Work: Access Crisis



Rural Utah residents are effectively in a care desert.

Source: American Board of Physical Therapy Specialties; U.S. Census

Specialty Care Gap

Patients Face Weeks of Delays

"Many reported reasonable wait-times for patients of a few days, however certain- such as pelvic health- could garner wait-times of up to a few weeks."

--Utah Department of Commerce Office of Professional Licensure Review, 2025 Periodic Review: Physical Therapy

Incontinence is a Health Hazard, Not Just a Nuisance.

A **low-risk** intervention (PT) prevents **high-cost** outcomes (Surgery/Depression).

(Milsom et al., 2019; Gallego-Gómez et al., 2024)

High-quality evidence shows that pelvic floor muscle training (PFMT) delivered via telehealth/tele-rehabiitation is feasible, effective, and in some cases comparable to in-person care for urinary incontinence and pelvic floor dysfunction.

Sources: Hao et al. (2024) systematic review & meta-analysis; Lima et al. (2025)

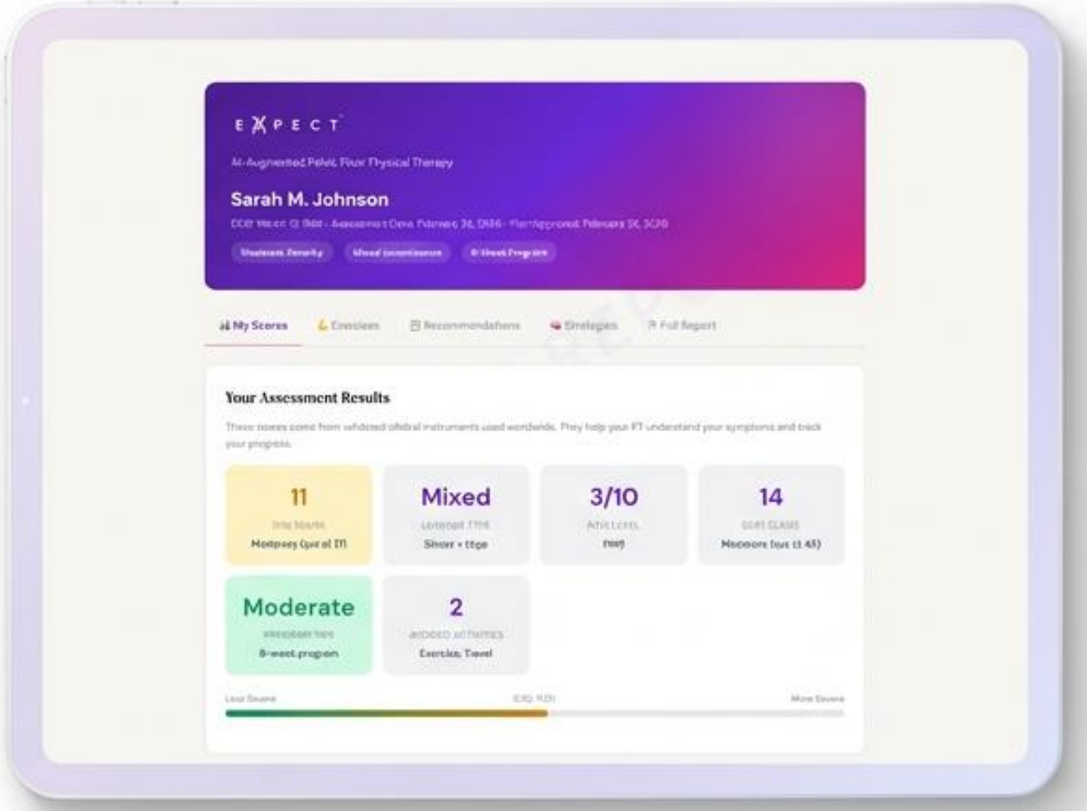
AI-Augmented Care for Pelvic Health

An AI-augmented platform for urinary incontinence, pelvic pain, and postpartum dysfunction.

Analysis: Uses LLM to analyze patient responses across validated instruments.

Scoring: Computes severity scores (ICIQ, GUPI) and classifications.

Planning: Generates PT-grade plans (ICD-10, exercises, precautions).



From Intake to Outcomes: The Patient Journey

Intake

→

Scoring

AI computes severity.

→

Draft

AI proposes plan.

→

Review (Gatekeeper)

Licensed Utah PT reviews, edits, and approves.

→

Delivery

→

Loop

Outcomes Audit.

EXPECT™

Hi Lisa,

You have a new document available in your secure Expect portal.

Sign in to View

For your security, this link will expire in 24 hours. If you did not request this, please ignore this email. Do not reply to this email. This inbox is not monitored.

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Risk Management & Mitigation

Risk: AI Failure

Mitigation: Mandatory PT review (Phase 1) & Validated Instruments.

Risk: Adverse Event

Mitigation: Red-flag triage, Pain precautions, “One Strike” rollback rule.

Risk: Data Privacy

Mitigation: HIPAA-compliant architecture, Encryption at rest.

Risk: Hallucination

Mitigation: Deterministic scoring algorithms (no generative text for diagnosis).

Safety First: The ‘Earned Autonomy’ Model

Modeled on the approved Doctronic precedent.

Calibration

100% PT Review.
0-250 cases per diagnosis.
Every plan human-verified.

→

Graduated Autonomy

Low-risk plans auto-approved
+ 10% Human Audit.
Only after statistical reliability.

Safety Net High-risk/Complex cases = Always 100% Human Review.

A 12-Month Controlled Pilot

Months 1-4

Months 1-4: Calibration

100% PT Review (Goal: 250 successful cases per diagnosis)

Months 5-8

Months 5-8: High-Frequency Sampling

Auto-approval for low-risk cases + 10% Audit

Months 9-12

Months 9-12: Expanded Deployment

Statewide Medicaid partnerships

Safety (Adverse events = 0)

Reliability (AI/PT Agreement >95%)

From Intake to Outcomes: The Patient Journey

Intake

→

Scoring

AI computes severity.

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Draft

AI proposes plan.

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Licensed Utah PT reviews, edits, and approves.

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Delivery

→

Loop

Outcomes Audit.

Best-in-Class Clinical Oversight

Dr. Nicole L. Dugan, PT, DPT, WCS

- **25+ years** of specialist experience.
- Board-Certified Women's Health Clinical Specialist (WCS).
- Former Penn Medicine Lead Physical Therapist
- Co-author of the **#9 most downloaded paper** in the history of the Journal of Women's & Pelvic Health Physical Therapy (2025).

The AI protocols are built on this gold-standard expertise.

Radical Transparency for the OAIP

- Real-time Compliance Dashboard
- Monthly Safety & Efficacy Reports
- De-identified datasets for researchers
- Public-facing Outcomes Dashboard

Enabling Innovation within Safe Bounds

Request 1

Interpretation: Recognition that AI-generated support under PT review is not “unlicensed practice”.

Request 2

Framework: Application of the “Earned Autonomy” framework (Doctronic precedent) for Phase 2.

Request 3

Safety Net: A 30-day cure period for inadvertent friction.

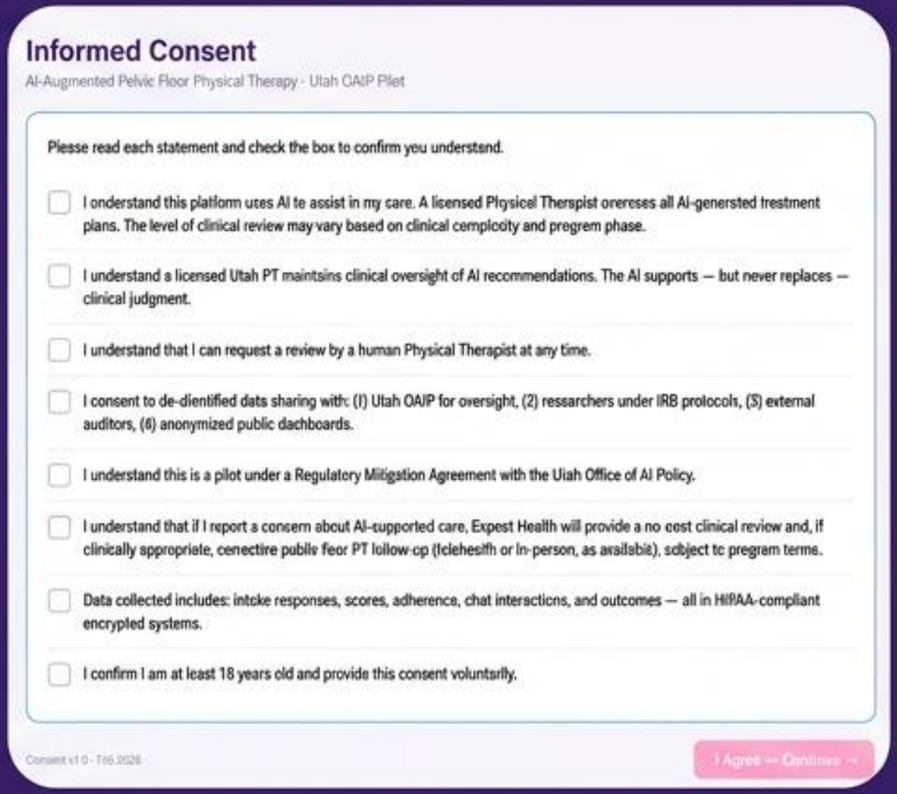
Feasible, Insured, and Ready for Responsible Pilot Deployment

Feasibility:

- Expertise (clinical + technical)
- Liability insurance in place
- Existing operating experience

Public Trust:

- Clear patient consent/disclosures
- Public reporting cadence
- Clinician oversight messaging



Informed Consent

AI-Augmented Pelvic Floor Physical Therapy · Utah OAIP Pilot

Please read each statement and check the box to confirm you understand.

- [ ] I understand this platform uses AI to assist in my care. A licensed Physical Therapist oversees all AI-generated treatment plans. The level of clinical review may vary based on clinical complexity and program phase.
- [ ] I understand a licensed Utah PT maintains clinical oversight of AI recommendations. The AI supports — but never replaces — clinical judgment.
- [ ] I understand that I can request a review by a human Physical Therapist at any time.
- [ ] I consent to de-identified data sharing with: (1) Utah OAIP for oversight, (2) researchers under IRB protocols, (3) external auditors, (4) anonymized public dashboards.
- [ ] I understand this is a pilot under a Regulatory Mitigation Agreement with the Utah Office of AI Policy.
- [ ] I understand that if I report a concern about AI-supported care, Expest Health will provide a no cost clinical review and, if clinically appropriate, connection to PT follow-up (telehealth or in-person, as available), subject to program terms.
- [ ] Data collected includes: intake responses, scores, adherence, chat interactions, and outcomes — all in HIPAA-compliant encrypted systems.
- [ ] I confirm I am at least 18 years old and provide this consent voluntarily.

Consent v1.0 · T06.2026

I Agree — Continue →

Approve Regulatory Mitigation Agreement

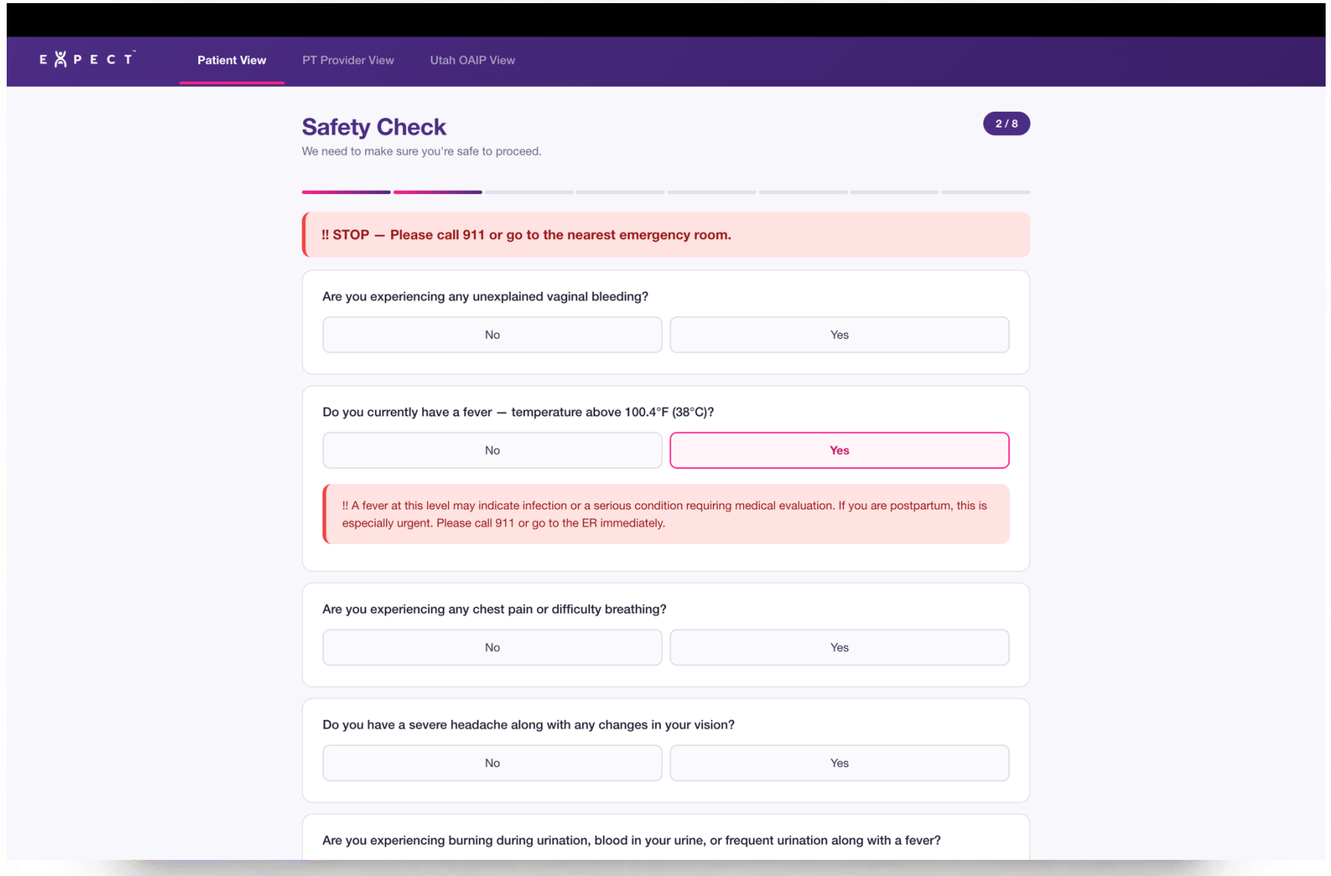
Ready to Scale Access for Utah Women

Safe. Effective. Regulated.

Pilot Authorization Request: February 2026

Contact: Dara Cook dara@expect.fit

Built to Exclude Risk



Red Flags: Fever, Bleeding, Chest Pain → Immediate redirect to ER/MD

Exclusions: Neurogenic bladder, Mesh complications, Grade 3+ Prolapse

The AI Role: The AI does not diagnose complex pathology; it applies a deterministic protocol to a validated score.

Doctor, Not Device.

Modeled on the approved Doctronic Sandbox precedent.

Phase 1: **Calibration**

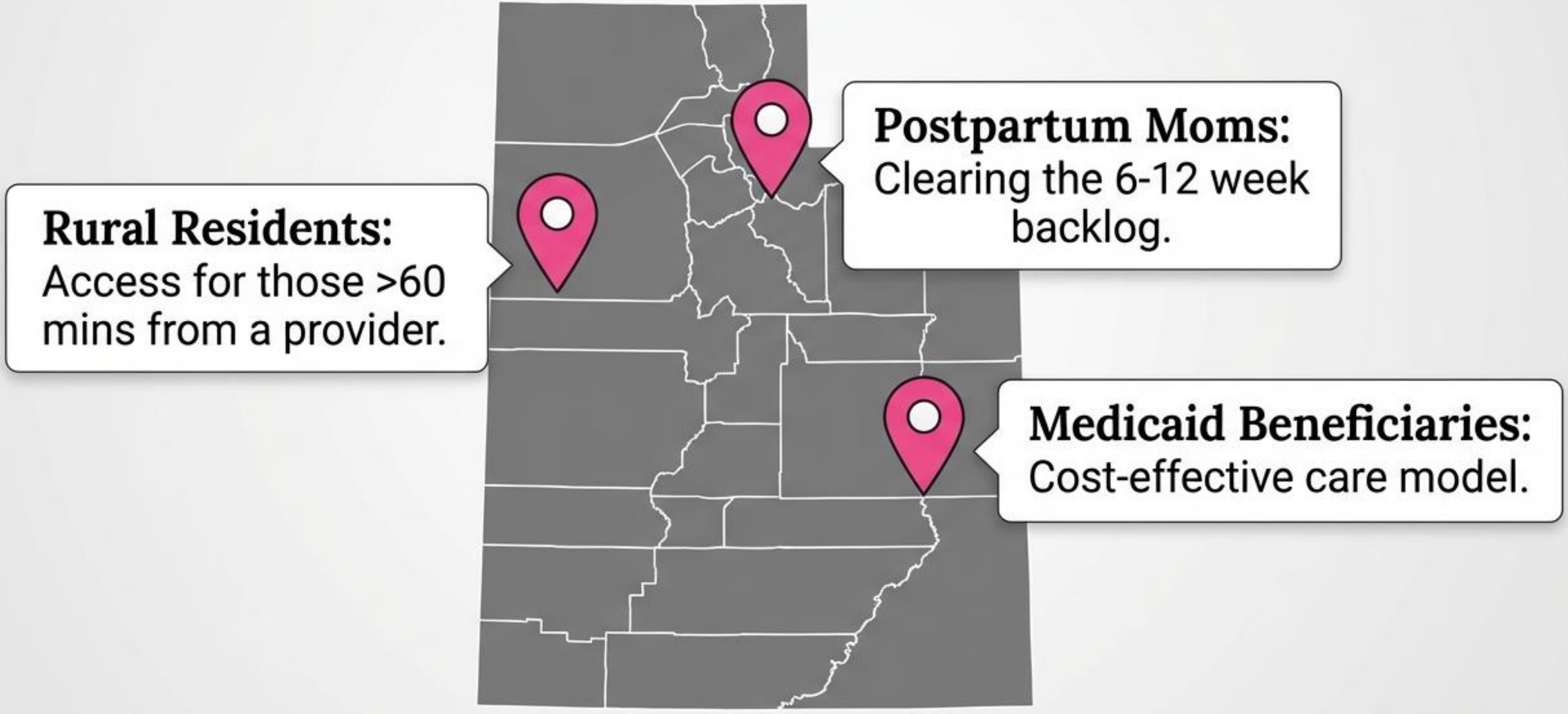
100% Human PT Review

Phase 2: **Validation**

Auto-approval for low-risk after 250+ cases

Safeguard 10% Ongoing Human Audit + 100% High-Risk Review

Scaling Access for Every Utahn



Built on Gold-Standard Clinical Data

ICIQ-UI SF	ICIQ-FLUTS	GUPI-Female	Pain Assessment
Global gold standard for incontinence severity.	Assesses filling and voiding symptoms.	Genitourinary Pain Index.	Standardized 0-10 mapping.

The AI does not ‘guess.” It processes standardized **clinical data points** used by specialists worldwide.

The Problem is Scale.

Pelvic floor symptoms are widespread, costly, and treatable with low-risk first-line training—but correct, consistent coaching doesn't scale with today's clinician capacity.

The Solution is Supervision.

AI is not replacing the clinician; AI is scaling the supervision required to treat the population.

Thesis: We must close the gap between massive need and scarce expertise.

Safety First: The ‘Earned Autonomy’ Model

Modeled on the approved Doctronic precedent.

Calibration

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Every plan human-verified.

→

Graduated Autonomy

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Safety Net High-risk/Complex cases = Always 100% Human Review.

Doctor, Not Device

A phased oversight model aligned with sandbox safety expectations.

Phase 1
(Calibration)

100% PT review before plan delivery.

Phase 2
(Validation)

Low-risk auto-approval after benchmark performance (250+ cases per subtype) + 10% human audit.

Phase 3
(Scale)

Continued OAIP data reporting and metrics.

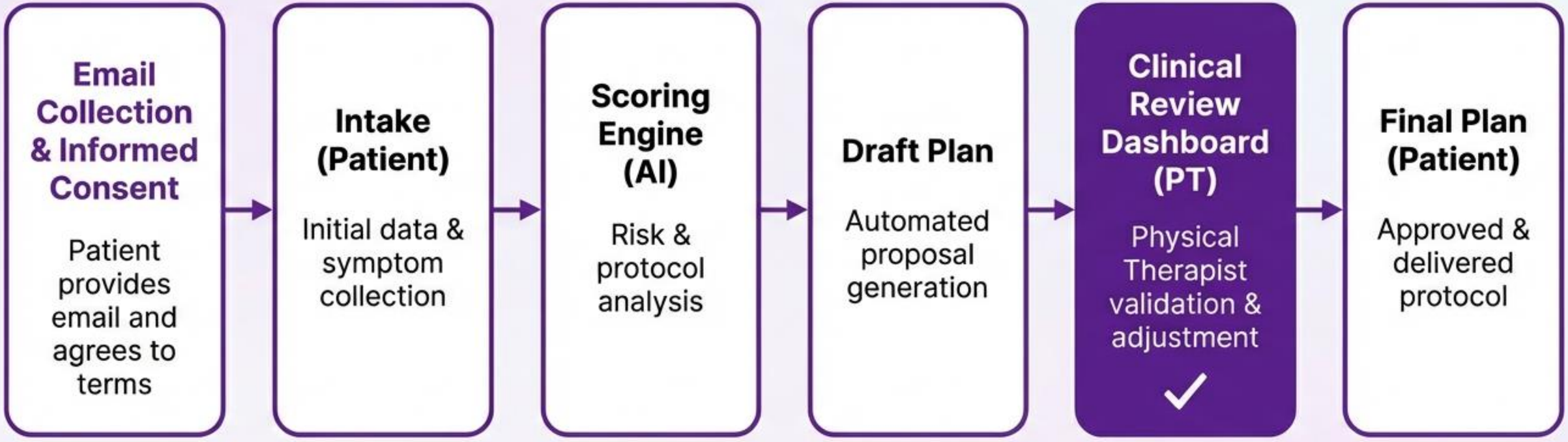
Knowing What We Don’t Treat: Exclusionary Criteria

Automatic Referral / Exclusion List

- ✕ Neurological conditions (MS, Parkinson’s)
- ✕ Grade 3+ Pelvic Organ Prolapse
- ✕ Post-surgical mesh complications
- ✕ Active infections/abscesses

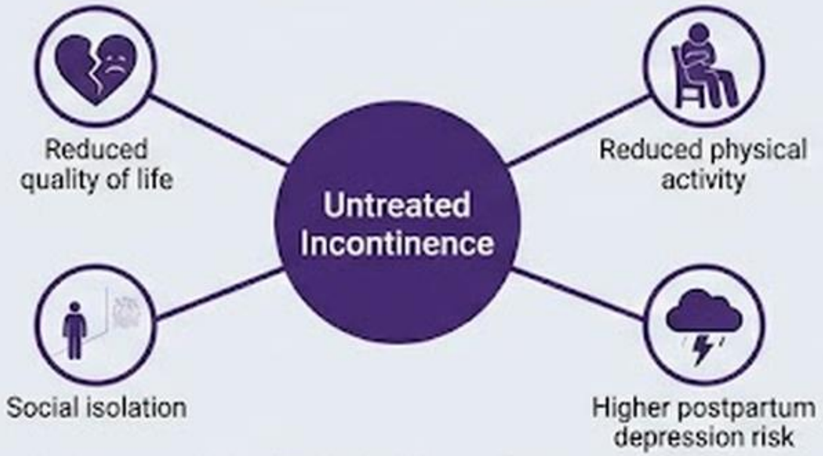
We reduce risk by defining a strict ‘Safe Harbor’ of exercise-responsive conditions.

The “Human-in-the-Loop” Workflow



The Cost of Inaction vs. The Safety of Intervention

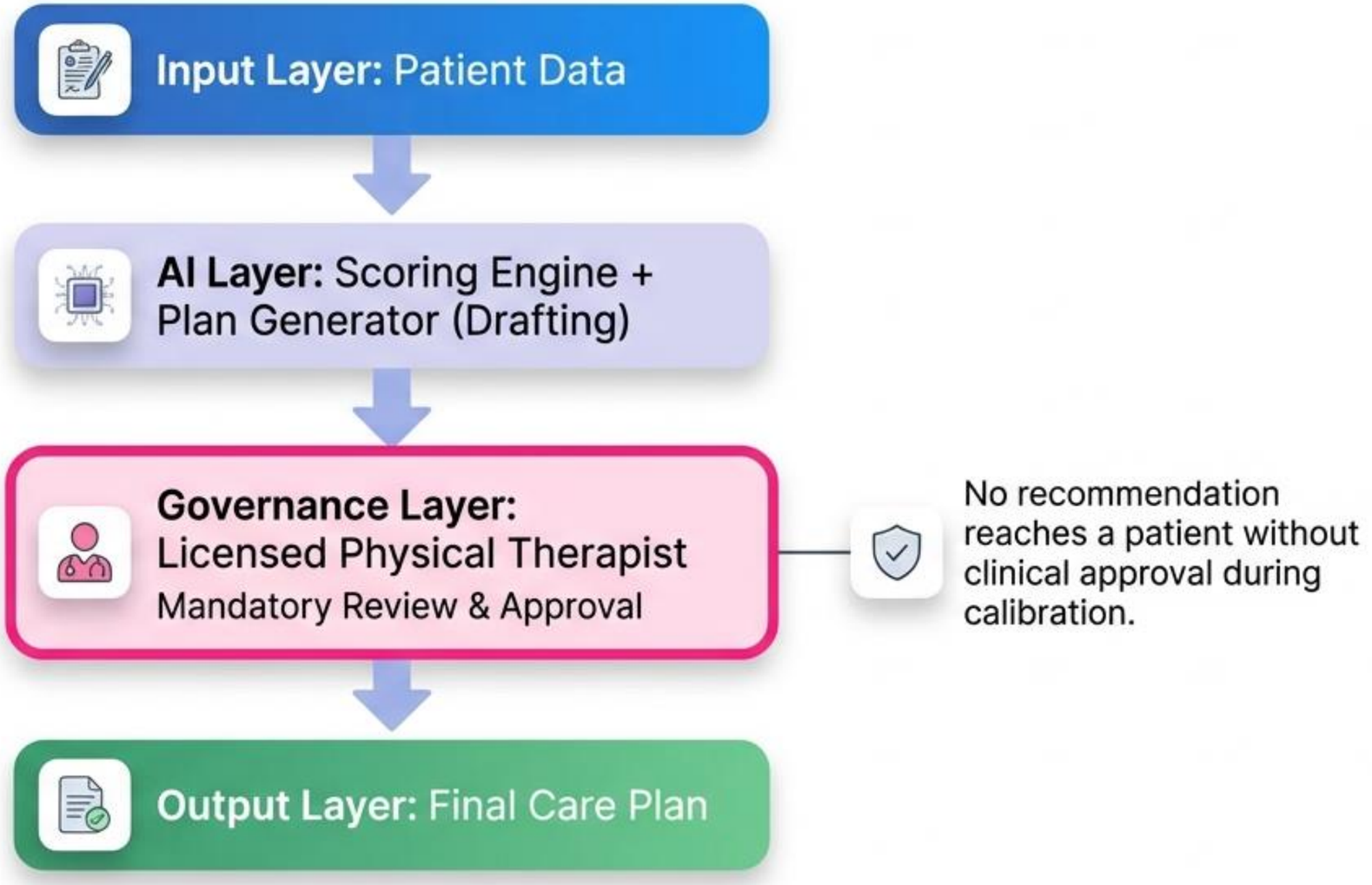
Harms of Inaction



(Milsom et al., 2019; Gallego-Gómez et al., 2024)

Low-Risk Intervention

- Conservative, non-invasive therapy
- Bounded adverse events (mechanical soreness)
- No systemic pharmaceutical toxicity

A **low-risk** intervention (PT) prevents **high-cost** outcomes (Surgery/Depression).

The ‘Doctor, Not Device’ Architecture